

St. Clair Hospital

1000 Bower Hill Rd, Pittsburgh, PA 15243

DISCHARGE SUMMARY Document #: DIS-018

Patient & Admission

Name	Tamara Higgins
Date of Birth	1970-10-04
Phone	412-555-5947
Address	374 Jackson Rest Greensburg, PA, 15601
MRN	MRN-496631
Admit Date	2026-05-07
Discharge Date	2026-05-12
Attending	Dakota Foley, MD
Attending NPI	4294019656

Insurance

Primary Payer	Cigna
Primary Member #	U531003309

Primary Discharge Diagnosis

Atrial flutter s/p cavotricuspid isthmus ablation

ICD-10: I48.0, Z95.0

Hospital Course

The patient presented with symptomatic atrial flutter refractory to rate-control medications and was admitted for electrophysiology evaluation and ablation. Baseline electrocardiography demonstrated typical counterclockwise atrial flutter with a ventricular rate in the 140s despite ongoing metoprolol and diltiazem therapy. Transthoracic echocardiography revealed normal left ventricular ejection fraction and no structural heart disease. The patient underwent successful cavotricuspid isthmus ablation on hospital day two under conscious sedation. Electrophysiology mapping confirmed typical right atrial flutter with critical conduction block achieved across the isthmus following radiofrequency ablation. The procedure was uncomplicated with no vascular complications or tamponade physiology noted.

Post-procedure course was notable for transient atrial ectopy on telemetry monitoring, which resolved spontaneously by hospital day three. The patient remained hemodynamically stable throughout the admission with heart rates maintained in the 60–80 range. Metoprolol was continued for rate control and additional nodal protection. Anticoagulation with apixaban was initiated given the recent arrhythmia history, and the patient was counseled on stroke risk reduction. Transthoracic echocardiography was repeated prior to discharge and showed preserved left ventricular function with no new wall motion abnormalities.

The patient was discharged on hospital day five in stable condition with excellent symptom resolution and no recurrent arrhythmias documented. He was instructed to continue current antiarrhythmic and rate-control medications, maintain anticoagulation, and follow up with cardiology in two weeks for post-ablation assessment and rhythm monitoring. Patient education emphasized the importance of medication adherence and reporting any recurrent palpitations or syncope. Close outpatient follow-up was arranged to assess long-term ablation success and arrhythmia recurrence risk.

Procedures Performed

Cavotricuspid isthmus ablation	CPT 93653
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Medication Changes

Metoprolol tartrate 25 mg BID	ADJUSTED
Amiodarone	STOPPED
Apixaban 5 mg BID	STARTED
Flecainide 100 mg BID	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	25 days
Urgency Tier	MEDIUM
Clinical Flags	groin access precautions, post-ablation anticoagulation required, blanking period — symptoms expected

Dictated and electronically signed by:
Dakota Foley, MD
NPI: 4294019656
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